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Rosácea

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O que é?

Rosácea é uma doença inflamatória crônica da pele. A afecção se manifesta principalmente no centro da face, mas pode expandir-se pelas bochechas, nariz, testa e queixo e afeta mais os adultos entre 30 e 50 anos. Embora as mulheres sejam mais suscetíveis, os homens desenvolvem as formas mais graves da enfermidade

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O que é?

Provavelmente, diversos fatores estão envolvidos no aparecimento da rosácea. Segundo a Sociedade Brasileira de Dermatologia, os mais importantes são: predisposição genética, alterações emocionais e hormonais, mudanças bruscas de temperatura, exposição solar, uso de bebidas alcoólicas, medicamentos vasodilatadores ou fotossensibilizantes, ingestão de alimentos muito quentes.

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O que causa?

- H. Pilory
- SIBO
- Sd. fúngica
- Hipersesibilidade ao glúten
- Lácteos
- Obesidade
- Menopausa
- Sol
- Álcool
- Vasodilatadores - pré treinos
- Alimentos picantes
- Doenças auto imunes - DM1, EM, AR, carcinoma basocelular

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Review > J Drugs Dermatol. 2011 Jun;10(6):614-20.

Inflammation in rosacea and acne: Implications for patient care

Alan B Fleischer Jr

PMID: 21961194

Abstract

Rosacea and acne are chronic inflammatory skin conditions that share an inflammatory pathogenesis, but clinically remain quite distinct. Although many have long assumed that these conditions are primarily infectious, emerging evidence suggests that inflammation plays a critical role in the pathogenesis of these disorders. Part of the innate immune system, the antimicrobial and proinflammatory cathelicidins, may be downregulated by both azelaic acid and sub antimicrobial doxycycline. In acne, the creation of papules, pustules and nodules is clearly mediated through immune mechanisms, and the antiinflammatory effects of retinoids play a key role in management. Recent observations help us understand in greater detail the role that inflammation plays in these two diseases, and the mechanisms by which commonly used medications exert their effect by modulating inflammatory processes. This review will present and synthesize recently acquired information as it relates to inflammatory acne and rosacea pathogenesis and clinical management.

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American Journal of Clinical Dermatology (2020) 21 (Suppl 1):S25–S35
https://doi.org/10.1007/s40257-020-00546-8

REVIEW ARTICLE

Microbiota in Rosacea

Hei Sung Kim[1,2,3]

Published online: 10 September 2020
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Abstract

Rosacea is a complex facial skin condition associated with abnormal inflammation and vascular dysfunction. Next to the known trigger factors, the role of microbiota in the development and aggravation of rosacea continues to raise interest. *Demodex folliculorum* mites, *Helicobacter pylori*, *Staphylococcus epidermidis*, *Chlamydia pneumoniae,* and the *Demodex*-associated bacterium, *Bacillus oleronius* are microbes that have been linked with rosacea. However, the results of studies which assessed their involvement in the disease have been inconsistent and inconclusive. Microbiological research in many different disciplines exploded in recent years as methods to analyze complex microbial communities at the taxonomic and phylogenetic levels became available. Here, we provide an update on the microorganisms implicated in rosacea and review the potential pathogenic role of microbes in the development of rosacea.

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Digestão

Sistema imunológico

Inflamação

Doença de pele

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Review > J Drugs Dermatol. 2020 Jun 1;19(6):585-591. doi: 10.36849/JDD.2020.5187.

Treating Inflammation in Rosacea: Current Options and Unmet Needs

Jerry Tan, J Mark Jackson

PMID: 32574018 DOI: 10.36849/JDD.2020.5187

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Abstract

Rosacea is a disease resulting from dysregulation of innate, adaptive, and neurovascular immune systems. Inflammatory pathways activated in rosacea can explain many of its signs and symptoms. Current treatments address some of these inflammatory processes, alleviating erythema and decreasing papules and pustules. However, for the majority of patients, complete clearance of these features is not currently achievable even with combination therapy. There is a need to address the spectrum of inflammatory processes involved in rosacea and for more efficacious agents with the goal of providing complete clearance for patients. J Drugs Dermatol. 2020;19(6): doi:10.36849/JDD.2020.5187.

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Review > Clin Exp Ophthalmol. 2021 Mar;49(2):104-117. doi: 10.1111/ceo.13900.
Epub 2021 Feb 3.

Ocular manifestations of rosacea: A clinical review

Shokufeh Tavassoli [1], Nathan Wong [1], Elsie Chan [1 2]

Affiliations + expand

PMID: 33403718 DOI: 10.1111/ceo.13900

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Abstract

Ocular rosacea is a chronic inflammatory condition that can occur in the absence of cutaneous features. The most common ocular features are chronic blepharoconjunctivitis with eyelid margin inflammation and meibomian gland dysfunction. Corneal complications include corneal vascularization, ulceration, scarring and, rarely, perforation. Diagnosis is largely based on clinical signs, although it is often delayed in the absence of cutaneous changes, particularly in children. It can also be associated with systemic disorders such as cardiovascular disease. Management ranges from local therapy to systemic treatment, depending on the severity of the disease. In this review, we describe the epidemiology, pathophysiology, clinical features and treatment of rosacea and ocular rosacea.

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Review > J Cosmet Dermatol. 2021 Sep;20(9):2715-2722. doi: 10.1111/jocd.13884.
Epub 2020 Dec 21.

Association between rosacea and cardiovascular disease: A systematic review and meta-analysis

Jiawen Zhang [1], Yang Yan [2], Peiyu Jiang [1], Zijing Liu [1], Yunyi Liu [1], Yixuan Liu [1], Xiaoyan Wang [3], Min Li [1], Yang Xu [1]

Affiliations + expand

PMID: 33350031 DOI: 10.1111/jocd.13884

Abstract

Background: Rosacea and cardiovascular diseases (CVD) are chronic inflammatory disorders. While CVD is the leading cause of mortality globally, increasing evidence indicates that CVD prevalence could be higher among patients with rosacea.

Aims: This review aimed to determine the association between the prevalence of CVD and rosacea.

Patients/methods: A systematic review of observational studies with controls available in MEDLINE, EMBASE, PubMed, Cochrane, and Web of Science databases was conducted. We performed a pooled meta-analysis using random-effects weighting. Overall, 11 studies met the inclusion criteria, which indicated increased odds for at least one risk factor of CVD, including diabetes, high blood pressure, or dyslipidemia.

Results: The pooled meta-analysis indicated an association of rosacea with higher odds of insulin resistance or diabetes (odds ratio [OR], 1.18; 95% confidence interval [CI], 0.97-1.45), high systolic blood pressure (OR, 1.96; 95% CI, 1.35-2.84), dyslipidemia (OR, 1.50; 95% CI, 1.19-1.88), and CVD (OR, 6.65; 95% CI, 2.80-15.76). No publication bias was detected. The effect of confounding factors due to overlapping symptoms and lack of individual-level data were limitations of this review.

Conclusion: Patients with rosacea have a high risk of CVD. However, further studies are warranted to confirm the association between rosacea and CVD.

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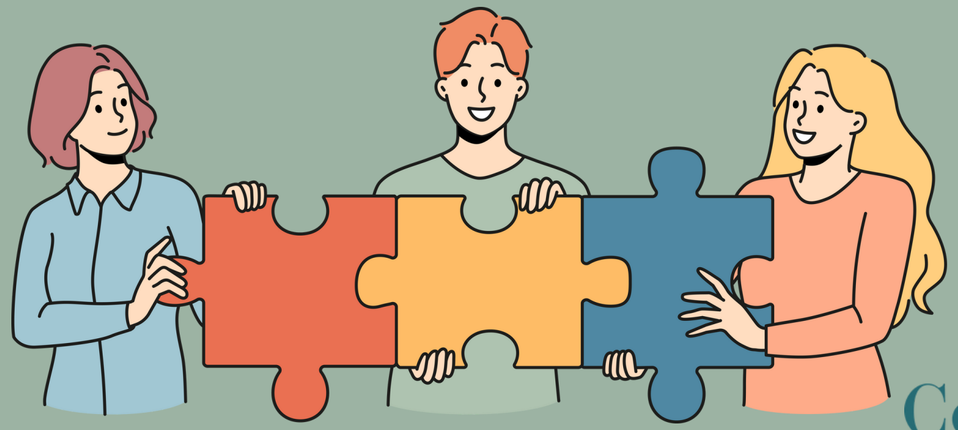

Review > Clin Dermatol. 2019 Nov-Dec;37(6):629-635.
doi: 10.1016/j.clindermatol.2019.07.032. Epub 2019 Jul 31.

Is rosacea a systemic disease?

Uwe Wollina [1]

Affiliations + expand

PMID: 31864441 DOI: 10.1016/j.clindermatol.2019.07.032

Abstract

Rosacea is a chronic inflammatory facial disease occurring world-wide. The incidence of rosacea is increasing with age, with the clinical course being characterized by relapses. The pathogenesis of rosacea is not completely understood, but neurovascular and immunologic mechanisms are involved. Rosacea has a number of known extrinsic triggers that should be avoided, such as sun exposure, heat and cold, alcoholic beverages, and spicy food. Of greater importance is the observation that rosacea may develop as a manifestation of systemic diseases with a significant morbidity and even mortality. Obesity, Helicobacter pylori infection, smoking, and inflammatory bowel disease bear a significant risk for the development of rosacea. Metabolic, psychiatric, and neurologic disorders and certain types of cancer show a significant association with rosacea. The possible link to cardiovascular events is debatable. There are extrafacial and extracutaneous manifestations of rosacea, such as the red scalp syndrome, ocular rosacea, and migraine. Rosacea should be considered a systemic disease.

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PRIMEIRO PASSO

Protocolo Desinflama Conecta

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ABSTRACT

OBJECTIVE: Rosacea is a common inflammatory cutaneous condition with a complex yet unknown etiopathogenesis. Diet and certain food items are known to trigger or worsen rosacea symptoms, but conflicting and often inconsistent advice is given to patients regarding this link. We provide an up-to-date literature review on the relationship between rosacea and diet. **METHODS:** Using the keywords "alcohol," "diet," "flushing," "food," "inflammation," "rosacea," "skin-gut axis" and "spice" we systemically searched the databases PubMed, MEDLINE and EMBASE for English-language articles in July 2020. **RESULTS:** The most frequently reported triggers implicated in rosacea include alcohol, spicy food, cinnamaldehyde-containing foods (e.g., tomatoes, citrus fruits, chocolate), hot drinks, and histamine-rich foods (e.g., aged cheese, wine, processed meats). Some food items appear to play a protective role, such as omega-3, which appears to protect against ocular rosacea. The relationship between certain food items and the subtype of rosacea is varied, with inconsistent results shown in the few studies that examined this. As an example, alcohol worsens flushing and fatty food triggers both erythematotelangiectatatic and phymatous rosacea in susceptible individuals. **CONCLUSION:** While several food types appear to be associated with exacerbation of rosacea, there are no recommendations that can be applied to all patients. Further studies are needed to examine the exact link between diet and rosacea subtypes.

KEYWORDS: Alcohol, diet, flushing, food, inflammation, rosacea, skin-gut axis, spice

Rosacea and Diet: What is New in 2021?

by TAMARA SEARLE, BSc, MBChB; FAISAL R. ALI , MA, PhD, BM BCH, MRCS, FRCP; SARAH CAROLIDES, MA, MPhil; and FIRAS AL-NIAIMI, MSc MRCP EBDV

Dr. Searle is with North Middlesex University Hospital in London, United Kingdom. Dr. Ali is with the Dermatological Surgery & Laser Unit in London, United Kingdom. Ms. Carolides is with COACH Medical inSt James's, London. Dr. Al-Niaimi is with the Department of Dermatology at Aalborg University Hospital in Aalborg, Denmark.

J Clin Aesthet Dermatol. 2021;14(12):49–54.

Rosacea is a chronic, relapsing, inflammatory cutaneous condition whose clinical features include skin sensitivity, flushing, centrofacial erythema, papules, pustules and hyperplasia of sebaceous glands. Four sub-phenotypes of rosacea have historically been described: erythematotelangiectatic (ETT), papulopustular (PPR), phymatous (PhR), and ocular rosacea. These subtypes often overlap in affected patients.[1] In this review, subtypes will be referred to based on the era of the included studies. However, the inadequacies of subtyping in rosacea must not be overlooked; more recent categorization in terms of phenotypic presentation have been proposed.[1]

Environmental and genetic predisposition play a role in the vascular and inflammatory aspects of the disease.[1] There is a long-standing recognition that certain food types may worsen rosacea. We present the current understanding of the relationship between diet and rosacea.[2]

METHODS

Using the keywords "alcohol," "diet," "flushing," "food," "inflammation," "rosacea," "skin-gut axis" and "spice," we searched databases PubMed, MEDLINE, and EMBASE to find the relevant literature. We excluded any articles that were not in English. Our review was conducted in July 2020, and the time period of evidence was collected from the inception of these databases until July 14, 2020. The level of evidence was evaluated and selected according to the highest level working our way downwards. Using the Oxford Centre of Evidence-Based Medicine 2011 guidance, we evaluated and listed the evidence based on its strength from level 1 to 5 with systematic reviews and meta analyses considered first, randomized controlled trials second, cascading down to weaker level of evidence, such as case series.

RESULTS

Our search yielded 44 studies, and 32 of these were deemed relevant for review. Our findings are summarized in Table 1.

DISCUSSION

Rosacea and diet. The role of diet as a "trigger" for rosacea is widely accepted. Triggers reported include: spicy food, cinnamaldehyde-containing foods, and alcohol. In a National

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CORRESPONDENCE: Firas Al-Niaimi, MSc MRCP EBDV; Email: firas55@hotmail.com

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Dieta com baixa carga glicêmica
Anti-inflamatória

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Review > J Cosmet Dermatol. 2022 Jan;21(1):85-98. doi: 10.1111/jocd.14436.
Epub 2021 Sep 26.

Role of nutritional supplements in selected dermatological disorders: A review

Kabir Sardana [1], Soumya Sachdeva [1]

Affiliations + expand

PMID: 34564936 DOI: 10.1111/jocd.14436

Abstract

Background: While a plethora of literature continues to be published on the role of nutritional agents both in lay press and indexed journals, the data is not on a firm footing and leaves the dermatologist in a quandry and the patient confused. The various agents include vitamins, minerals, amino acids, antioxidants, diets & gluten. A proper knowledge of the role of nutritional supplements in dermatological diseases can be a useful tool in advising the patients and in certain cases ameliorating the disorder.

Patients/methods: Literature review of last 15 years was made using the terms "diet in dermatology," "nutrition and skin," "nutritional supplements in dermatology," "nutritional agents and acne," "nutritional agents and alopecia," and "nutritional agents and psoriasis."

Results: While there are multiple publications on the use of nutritional supplements for amelioration of skin diseases, most of them are based on either associations or in vitro studies, but very few transcend the rigors of a clinical trial or the holey grail of a double-blinded randomized controlled trial. There seem to be some evidence in acne, psoriasis, telogen effluvium, urticaria & vitiligo. Coeliac disease and dermatitis herpetiformis have a strong link with diet. Rosacea has a strong link with certain foods, but the other disorders like melasma, aphthous stomatitis do not have any scientifically validated association with diet.

Conclusions: Our updated review examines the role of nutritional supplements and antioxidants in various dermatological disorders. We have found that there are varying levels of evidence with notable associations of low glycemic diet & acne, fish oil & weight loss with psoriasis, fish oils & probiotics with atopic dermatitis & vitamins & botanical extracts with vitiligo. The evidence for diet and nutrition in bullous disorders and photoageing is scarce. The role of low histamine diet in urticaria is useful in select cases of episodic urticaria. Rosacea is triggered by hot and spicy food . Apart from gluten and Dermatitis Herpetiformis, no diet can be considered disease modifying in our reveiw. The lack of comparison of nutritional or dietary modiffication with conventional validated agents, makes the data difficult to translate in real world patient management.

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Suplementos

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CSC

- Vit. A, ácido fólico, C
- Vit. K, Fe, Ca, Zn
- Ômega 3
- Nicotinamida
- Chá verde, camomila, alcaçuz, lavanda
- Resveratrol
- Curcumina
- Tanaceto
- Polypodium leucotomos

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CSC

- Polifenóis
- Silimarina
- Quassia
- Aveia
- Óleo de cânfora

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Review > J Cosmet Dermatol. 2014 Dec;13(4):324-8. doi: 10.1111/jocd.12119.

A review of nicotinamide: treatment of skin diseases and potential side effects

Heidi M Rolfe [1]

Affiliations + expand

PMID: 25399625 DOI: 10.1111/jocd.12119

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Abstract

Nicotinamide, also known as niacinamide, is the amide form of vitamin B3. It is a precursor of essential coenzymes for numerous reactions in the body including adenosine triphosphate (ATP) production. Nicotinic acid, also known as niacin, is converted into nicotinamide in the body. The use of topical nicotinamide in the treatment of acne vulgaris; melasma; atopic dermatitis; rosacea; and oral nicotinamide in preventing nonmelanoma skin cancer is discussed. The possible side effects and consequences of excessive nicotinamide exposure are reviewed, including suggestions nicotinamide might have a role in the development of diabetes, Parkinson's disease, and liver damage.

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Review > Am J Clin Dermatol. 2015 Feb;16(1):27-33. doi: 10.1007/s40257-014-0107-3.

A review of vitamin B12 in dermatology

Jennifer Brescoll [1], Steven Daveluy

Affiliations + expand

PMID: 25559140 DOI: 10.1007/s40257-014-0107-3

Abstract

Vitamin B12, also known as cobalamin, is a water-soluble vitamin that is important in the hematological and nervous systems, and it has a complex relationship with the skin. Altered cobalamin levels can lead to dermatological manifestations, which may indicate a deficiency or excess of this vitamin. The biochemistry and metabolism of cobalamin is complex, and diseases can be associated with alterations of this metabolic pathway. The cutaneous manifestations of cobalamin deficiency include hyperpigmentation (most commonly), hair and nail changes, and oral changes, including glossitis. Additionally, several dermatologic conditions, including vitiligo, aphthous stomatitis, atopic dermatitis, and acne are related to cobalamin excess or deficiency. The cutaneous complications of cobalamin therapy include acne, rosacea, and allergic site reactions, or anaphylaxis with cobalamin injections. As cobalt is a component of cobalamin, patients with cobalt sensitivity have been reported to have cutaneous manifestations when receiving cobalamin replacement therapy.

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Valor ideal : 500

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Case Reports > J Eur Acad Dermatol Venereol. 2001 Sep;15(5):484-5.
doi: 10.1046/j.1468-3083.2001.00308.x.

Rosacea fulminans triggered by high-dose vitamins B6 and B12

T Jansen [1], R Romiti, A Kreuter, P Altmeyer

Affiliations + expand

PMID: 11763399 DOI: 10.1046/j.1468-3083.2001.00308.x

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Abstract

Rosacea fulminans is a rare variant of rosacea conglobata that occurs almost exclusively in women well past adolescence. The aetiology is unknown, although immunological, hormonal, and vascular factors have been suggested. We report the case of a 17-year-old girl with rosacea fulminans that was temporally associated with daily ingestion of high-dose vitamin B supplements. The onset was sudden and cosmetically disabling. The eruption improved when the vitamin supplement was discontinued and a therapeutic regimen including isotretinoin and methylprednisolone was introduced. It seems appropriate to consider the possibility of such a vitamin B-triggered condition in cases of subjects presenting new or exacerbating facial eruptions.

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J Clin Aesthet Dermatol. 2022 Jun;15(6):42-45.

Hydroxocobalamin: An Effective Treatment for Flushing and Persistent Erythema in Rosacea

Yi-Wei Huang [1], Hui-Peng Huang [2], Chao-Kai Hsu [3], Julia Yu-Yun Lee [3]

Affiliations + expand

PMID: 35783562 PMCID: PMC9239126

Free PMC article

Abstract

Background: Expression of inducible nitric oxide synthase (NOS) is higher in rosacea skin samples than in normal skin controls. Hydroxocobalamin is a potent inhibitor of all isoforms of NOS, capable of reducing the vasodilatations induced by nitric oxide.

Objective: We aimed to evaluate the role of hydroxocobalamin in treating facial flushing and persistent erythema of rosacea.

Methods: Thirteen patients with rosacea who displayed facial flushing and persistent erythema received 1 to 4 weekly intramuscular injections of hydroxocobalamin 1 to 2 mg. The outcomes were measured using the Clinician's Erythema Assessment (CEA) by photography and an infrared thermometer to evaluate the difference in skin surface temperature (SST) of the cheeks before and after treatment.

Results: Thirty minutes after the first dose of intramuscular injection of hydroxocobalamin, the mean CEA significantly reduced from 2.2± 0.6 to 1.2±0.4 (p<0.001), and average SST also significantly reduced from 36.7±0.70°C to 36.2±0.61°C (p<0.001) on the cheeks.

Conclusion: In our patient sample, intramuscular administration of hydroxocobalamin was effective for immediate reduction of facial erythema associated with rosacea.

Keywords: Hydroxocobalamin; erythema; flushing; nitric oxide synthase; rosacea; vitamin B12.

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> Cutan Ocul Toxicol. 2014 Mar;33(1):60-2. doi: 10.3109/15569527.2013.797907.
Epub 2013 May 28.

Vitamin D status in patients with rosacea

Ozlem Ekiz [1], Ilknur Balta, Bilge Bülbül Sen, Meltem Cik Dikilitaş, Pınar Ozuğuz, Emine Nur Rifaioğlu

Affiliations + expand

PMID: 23713748 DOI: 10.3109/15569527.2013.797907

Abstract

Background: Rosacea is a common chronic skin condition affecting the face. In recent years, significant evidence shows that vitamin D plays an important role in modulating the immune system. Vitamin D and its analogues via these mechanisms are playing an increasing role in the management of atopic dermatitis, psoriasis, vitiligo, acne and rosacea.

Objectives: In our study, we aimed to investigate the relationship between serum vitamin D levels in patients with rosacea and analyze the association of vitamin D with clinical features.

Methods: Forty-four rosacea patients and 32 healthy control subjects were included into the study. 25-hydroxyvitamin D (25(OH)D), calcium and intact parathyroid hormone were measured. Deficiency of vitamin D is defined as the level of 25(OH)D being less than 20 ng/ml.

Results: Thirty-three female and 11 male patients were included in the study. The mean age of patients was 48.6 ± 11.5. The mean levels of vitamin D levels were found as 21.4 ± 9.9 and 17.1 ± 7.9 in patients and controls, respectively. The difference was statistically significant (p = 0.04). The prevalence of vitamin D deficiency in patients with rosacea was 38.6% and 28.1% in healthy controls (p = 0.34).

Conclusions: To the best of our knowledge, our study is the first study for evaluating serum vitamin D levels of patients with rosacea in the literature. Patients with rosacea have relatively high serum vitamin D levels compared to control groups. The result of our study suggests that increased vitamin D levels may lead to the development of rosacea. To confirm status of vitamin D levels in patients with rosacea, larger epidemiological studies are needed.

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Valor ideal - 40 a 80

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Review > J Drugs Dermatol. 2018 Jun 1;17(6):611-617.

A Biologically Based Approach to Acne and Rosacea

Penelope J Kallis, Alexandra Price, Jacquelyn R Dosal, Anna J Nichols, Jonette Keri

PMID: 29879248

Abstract

Complementary and alternative medicine (CAM) therapies are increasing in popularity in the field of dermatology. Natural products and holistic approaches are in high demand among patients and research has begun to support their roles in acne and rosacea pathophysiology. In this article, commonly utilized biologically based complementary and alternative therapies for acne and rosacea are reviewed from an evidence-based perspective. Therapies discussed include vitamin C, nicotinamide, zinc, tea tree oil, green tea, resveratrol, curcumin, feverfew, licorice, chamomile, polypodium leucotomos, and nutrition-based approaches. J Drugs Dermatol. 2018;17(6):611-617.

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Informações Nutricionais

Dose: 2 Cápsulas Vegetarianas

Doses por Recipiente: 60

	Quantidade por Dose	% Valores Diários
Vitamina C (como ácido ascórbico da Red Orange Complex®)	5,5 mg	6%
Ácido Nicotínico (como Nicotinamida)	500 mg•	3.125%
Extrato de Polypodium leucotomos (folha)	240 mg	**
Red Orange Complex® [Laranjas vermelhas da Sicília (Citrus sinensis var. Extrato de Moro, Sanguinello, Tarocco) (fruta e casca)]	100 mg	**

** Valores Diários não estabelecidos. • NE (equivalentes de niacina).

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Review > Skinmed. 2012 Nov-Dec;10(6):352-5.

Green tea in dermatology

Nader Pazyar [1], Amir Feily, Afshin Kazerouni

Affiliations + expand

PMID: 23346663

Abstract

The purpose of this brief review is to summarize all in vitro, in vivo, and controlled clinical trials on green tea preparations and their uses in dermatology. An extensive literature search was carried out to identify in vivo and in vitro studies as well as clinical trials. Twenty studies were assessed and the results suggest that oral administration of green tea can be effective in the scavenging of free radicals, cancer prevention, hair loss, and skin aging plus protection against the adverse effects associated with psoralen-UV-A therapy. Topical application of green tea extract should be potentially effective for atopic dermatitis, acne vulgaris, rosacea, androgenetic alopecia, hirsutism, keloids, genital warts, cutaneous leishmaniasis, and candidiosis. There are promising results with the use of green tea for several dermatologic conditions; however, the efficacy of oral and topical green tea has not always been confirmed.

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Chá Verde

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Review > J Drugs Dermatol. 2006 Jan;5(1):29-32.

Treatment of rosacea with herbal ingredients

Jessica Wu [1]

Affiliations + expand

PMID: 16468289

Abstract

Since rosacea is a chronic disease and many patients find prescription therapies unsatisfactory, they frequently turn to herbal ingredients for relief of their persistent facial redness. The most useful and frequently used herbal compounds include licorice, feverfew, green tea, oatmeal, lavender, chamomile, tea tree oil, and camphor oil. The utility of most of these herbs is based on their purported anti-inflammatory properties. Some of these herbs have proven effects, many have potential benefits, and some may aggravate rosacea. Due to the fact that many patients fail to inform their physicians about their use of herbal ingredients, dermatologists should be aware of what patients may be using and be able to advise them about the efficacy of these ingredients or the potential for adverse effects.

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Alcaçuz, matricária, chá verde, aveia, lavanda, camomila, óleo de melaleuca e óleo de cânfora

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Meta-Analysis > Clin Exp Dermatol. 2021 Jul;46(5):825-833. doi: 10.1111/ced.14584.
Epub 2021 Mar 15.

Dermatological effects of Curcuma species: a systematic review

S M Barbalho [1 2 3], H F de Sousa Gonzaga [4], G A de Souza [2], R de Alvares Goulart [1], M L de Sousa Gonzaga [4], B de Alvarez Rezende [2]

Affiliations + expand

PMID: 33522006 DOI: 10.1111/ced.14584

Abstract

Curcuma and its derivatives are associated with anti-inflammatory and antioxidant activities in the skin. They exhibit beneficial effects in wound healing and prevention of chronic ultraviolet B damage and may prevent facial redness such as rosacea and flushing. This review aims to provide an up-to-date and rigorous synthesis of studies that demonstrated the clinical efficacy of curcuminoids in the skin. We evaluated studies published in the MEDLINE-PubMed/PMC (National Library of Medicine) databases, and followed Preferred Reporting Items for Systematic Reviews and Meta-Analyses guidelines for this review. This search included papers published in the past 10 years in controlled clinical trials, double-blind and randomized controlled studies, and case studies. The search resulted in 12 studies published in the past 10 years. Curcuma species (Curcuma longa and Curcuma aeruginosa) and curcumin were found to produce various dermatological effects, including influencing antioxidant and anti-inflammatory processes in the production of hyaluronan, increasing skin moisture, and reducing axillary hair growth. Curcuma was also found to reduce thickness, erythema, pruritus, burning and pain in psoriasis lesions and to improve radiodermatitis lesions. Our review results show that Curcuma species may play a role in skin health management and may exhibit various dermatological effects, thus it could be a new therapeutic arsenal for dermatology professionals. Nevertheless, more clinical trials should be conducted with humans to establish the optimum delivery method and dosages for different dermatological conditions.

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Licenciado para Lays Martins 074.232.516-40

J Drugs Dermatol. 2008 Jul;7(7 Suppl):s13-6.

Anti-inflammatory ingredients

Jessica Wu [1]

Affiliations + expand

PMID: 18681154

Abstract

There is a growing public awareness and concern among individuals regarding the condition of their skin, with a concomitant desire to use natural products to treat skin conditions. The increased interest in these products has spurred scientific and clinical studies evaluating the composition and clinical usefulness of natural products in the treatment of inflammatory skin dermatoses. There are numerous natural ingredients that have been demonstrated to possess anti-inflammatory properties that make formulations containing these ingredients attractive treatment options. This article summarizes the active ingredients, anti-inflammatory properties, clinical effects, and therapeutic potential of colloidal oatmeal, feverfew, licorice, aloe vera, chamomile, and turmeric. Potential therapeutic indications include erythema induced by ultraviolet light, rosacea, atopic dermatitis, sensitive and irritated skin, drug-induced skin eruptions, and psoriasis. These products may be particularly well suited as alternatives to pharmacologic therapies in chronic conditions for which long-term use is required.

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Randomized Controlled Trial > J Cosmet Dermatol. 2008 Mar;7(1):8-14.
doi: 10.1111/j.1473-2165.2008.00355.x.

Combined effects of silymarin and methylsulfonylmethane in the management of rosacea: clinical and instrumental evaluation

Enzo Berardesca [1], Norma Cameli, Claudia Cavallotti, Jean Luc Levy, Gérald E Piérard, Gianfranco de Paoli Ambrosi

Affiliations + expand

PMID: 18254805 DOI: 10.1111/j.1473-2165.2008.00355.x

Abstract

Objective: This study aims to evaluate a topical treatment based on silymarin/methylsulfonilmethane (S-MSM) to improve erythematous-telangiectactic rosacea.

Methods: Forty-six patients affected by stage I-III rosacea entered this double-blind, placebo-controlled study. Subjects were treated for 1 month. Clinical and instrumental evaluations were done at baseline, after 10 and 20 days, and at the end of the study. Itching, stinging, erythema, and papules were investigated clinically as well as hydration and erythema instrumentally with capacitance and color measurements.

Results: A statistically significant improvement was observed in many clinical and instrumental parameters investigated ($P < 0.001$). In particular, improvement of skin redness, papules, itching, hydration, and skin color occurred.

Conclusions: The combination of silymarin and S-MSM can be useful in managing symptoms and condition of rosacea skin, especially in the rosacea subtype 1 erythemato-telangiectatic phase. The action can be considered multicentric and multiphase because of the direct modulating action on cytokines and angiokines normally involved and up-regulated in the case of such skin condition.

Jarrow Formulas, MSM, 1.000 mg, 200 Cápsulas Vegetais

Doctor's Best, MSM com OptiMSM, 1.000 mg, 180 Cápsulas Vegetais

Now Foods, MSM, Metilsulfonilmetano, 1.000 mg, 240 Cápsulas Vegetais

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Review > Am J Lifestyle Med. 2021 Jul 2;17(1):152-160. doi: 10.1177/15598276211026592. eCollection 2023 Jan-Feb.

Diet in Dermatology: Review of Diet's Influence on the Conditions of Rosacea, Hidradenitis Suppurativa, Herpes Labialis, and Vitiligo

Marielle Jamgochian [1], Mahin Alamgir [1], Babar Rao [1]

Affiliations + expand

PMID: 36636389 PMCID: PMC9830249 DOI: 10.1177/15598276211026592

Free PMC article

Abstract

The influence of dietary patterns on cutaneous disease has been an oft-posed question to dermatologists by patients in a clinical setting. Similarly, the popularity of nutritional supplementation with vitamins, minerals, and nutraceutical blends has been increasing. Dermatologists, primary care physicians, and other providers should be familiar with dietary interventions that are evidence-based and those that are more marketable than efficacious. In this review, the modification of diet, including dietary exclusion and dietary supplementation for the treatment of rosacea, hidradenitis suppurativa (HS), herpes labialis, and vitiligo was investigated. Despite abundant anecdotal evidence, the literature search found no high-quality evidence that an elimination diet for rosacea "trigger foods" improved rosacea symptoms though these elimination diets (of hot, spicy, alcohol-containing, or cinnamaldehyde-containing foods) had low risk of harm. There is evidence that zinc supplementation and vitamin D supplementation in deficient patients is helpful for treating HS. For herpes labialis, L-lysine supplementation was found to be effective for prophylaxis but not for decreasing duration of active lesions. For vitiligo, the use of the herb *Polypodium leucotomos* in conjunction with phototherapy was found to increase repigmentation, as well as vitamin D supplementation in deficient patients.

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Como usar ?

1. Escolher DOIS suplementos para ROSÁCEA
2. Pode acrescentar esses dois aos escolhidos de outras imersão
3. Usar na dose indicada do fabricante
4. Utilizar por até 3 meses e rotacionar
5. Pode tomar com chá de matricária, feno grego, bardana ou verde

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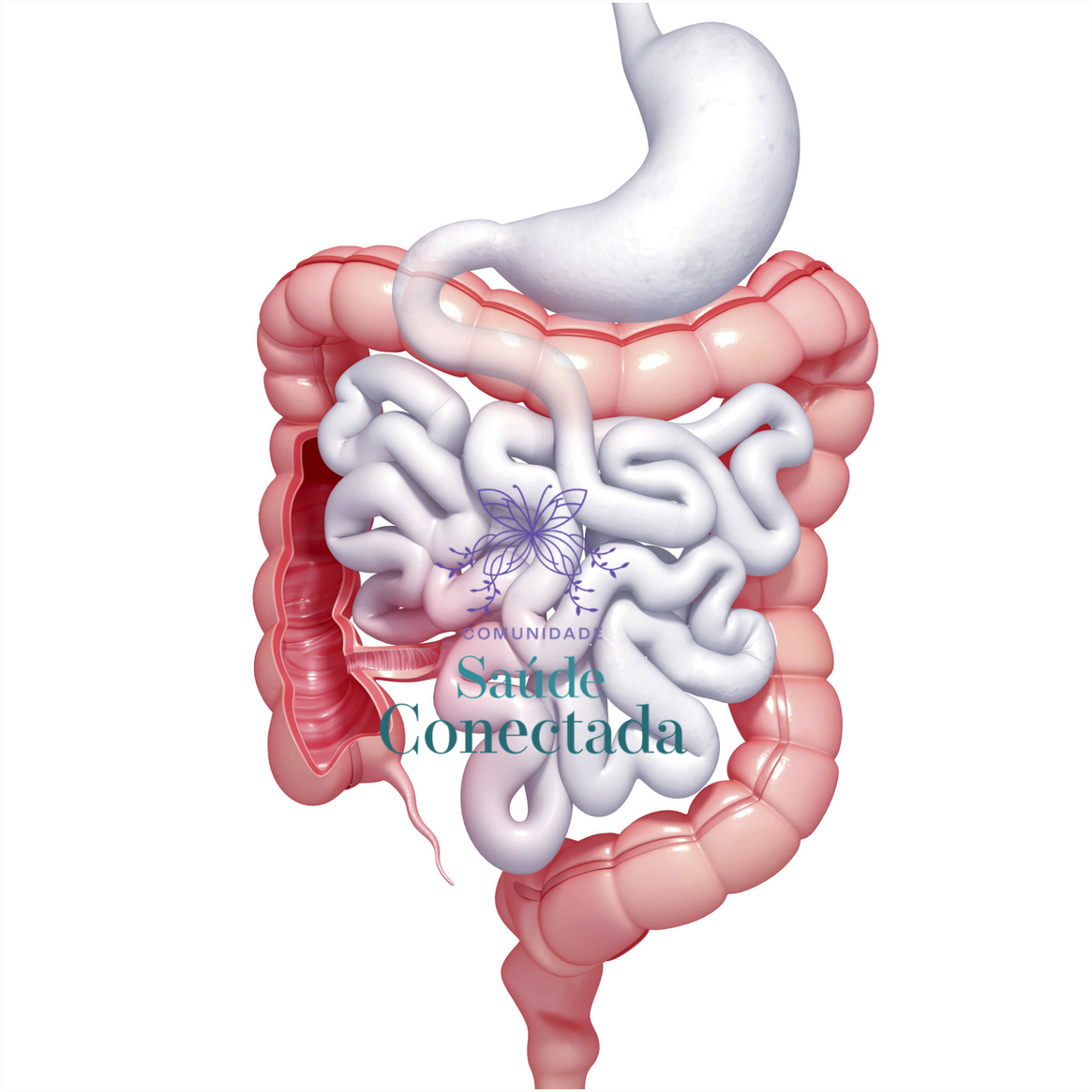

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Microorganisms. 2019 Nov; 7(11): 550.
Published online 2019 Nov 11. doi: 10.3390/microorganisms7110550
PMCID: PMC6920876
PMID: 31717915

The Skin and Gut Microbiome and Its Role in Common Dermatologic Conditions

Samantha R. Ellis,[1,2] Mimi Nguyen,[3] Alexandra R. Vaughn,[2] Manisha Notay,[2] Waqas A. Burney,[2,4] Simran Sandhu,[3] and Raja K. Sivamani[2,4,5,6,7,*]

Author information Article notes Copyright and License information Disclaimer

Abstract

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Microorganisms inhabit various areas of the body, including the gut and skin, and are important in maintaining homeostasis. Changes to the normal microflora due to genetic or environmental factors can contribute to the development of various disease states. In this review, we will discuss the relationship between the gut and skin microbiome and various dermatological diseases including acne, psoriasis, rosacea, and atopic dermatitis. In addition, we will discuss the impact of treatment on the microbiome and the role of probiotics.

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Review > Microorganisms. 2019 Nov 11;7(11):550. doi: 10.3390/microorganisms7110550.

The Skin and Gut Microbiome and Its Role in Common Dermatologic Conditions

Samantha R Ellis [1 2], Mimi Nguyen [3], Alexandra R Vaughn [2], Manisha Notay [2], Waqas A Burney [2 4], Simran Sandhu [3], Raja K Sivamani [2 4 5 6 7]

Affiliations + expand

PMID: 31717915 PMCID: PMC6920876 DOI: 10.3390/microorganisms7110550

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Abstract

Microorganisms inhabit various areas of the body, including the gut and skin, and are important in maintaining homeostasis. Changes to the normal microflora due to genetic or environmental factors can contribute to the development of various disease states. In this review, we will discuss the relationship between the gut and skin microbiome and various dermatological diseases including acne, psoriasis, rosacea, and atopic dermatitis. In addition, we will discuss the impact of treatment on the microbiome and the role of probiotics.

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Dermato-Endocrinology 1:5, 275-279; September/October, 2009; © 2009 Landes Bioscience

Probiotics for photoprotection

Audrey Guéniche,[1,*] David Philippe,[2] Philippe Bastien,[1] Stephanie Blum,[2] Elif Buyukpamukcu[2] and Isabelle Castiel-Higounenc[1]

[1]L'Oréal Recherche; Centre Charles Zviak; Clichy, France; [2]Nestle Research Center; Lausanne, Switzerland

Key words: *Lactobacillus johnsonii*, skin, immune homeostasis, stress, food supplement, probiotics, photoprotection

Specific strains of probiotic, have been identified as beneficial to influence the composition and/or metabolic activity of the endogenous microbiota and some of these strains have been also shown to inhibit the growth of a wide range of enteropathogens. The first aim of using probiotics has been to improve the composition of the intestinal microbiota from a potentially harmful composition towards a composition that would be beneficial to the host.

Beyond their capacity to influence positively the composition of the intestinal microbiota, several lines of evidence suggest that some probiotic bacteria can modulate the immune system both at the local and systemic levels thereby improving immune defense mechanisms and/or downregulate immune disorders such as allergies or intestinal inflammation.

Skin reflects the general health status and aging. Different human trials widely suggest that probiotic supplementation might be useful in the management of atopic dermatitis. Based on these properties it appears that, beyond the gut, probiotics might exert their benefits at the skin level.

In a randomized double blind placebo-controlled clinical trial, we investigated whether the probiotic bacteria *Lactobacillus johnsonii* NCC 533 (La1) could modulate the cutaneous immune homeostasis altered by solar-simulated UV exposure in humans. After, UV exposure to twice 1.5 MED, we demonstrated that La1 intake facilitated an earlier recovery of Epidermal cells allostimulatory function. Thus, this clinical data strengthen the assumption that certain probiotics can contribute to modulate skin immune system leading to the preservation of the skin homeostasis. Altogether the data affords the possibility of designing new strategies based on a nutritional approach for the prevention of UV-induced damaging effects.

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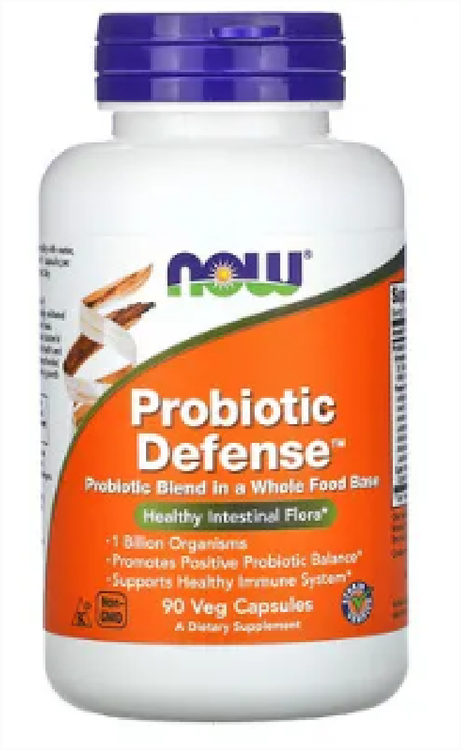
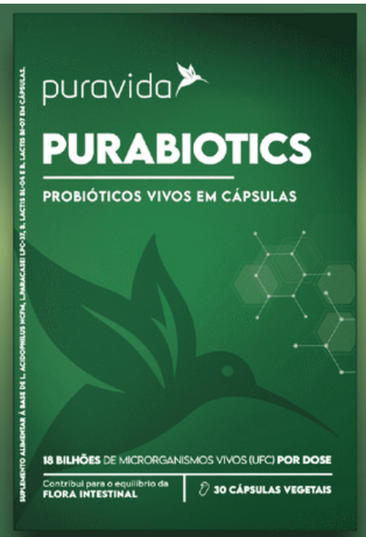


INGREDIENTES

Lactobacillus acidophilus NCFM (ATCC SD5221), Lactobacillus paracasei Lpc-37 (ATCC SD5275), Bifidobacterium animalis subsp lactis BI-04 (ATCC SD5219), Bifidobacterium animalis subsp lactis Bi-07 (ATCC SD5220), agente de massa celulose microcristalina, antiumectantes estearato de magnésio e dióxido de silício, cápsula (espessante hidroxipropilmetilcelulose).

NÃO CONTÉM GLÚTEN.



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INFORMAÇÃO NUTRICIONAL

Porção de 1,9 g (3 cápsulas)

Quantidade por porção		%VD
Lactobacillus acidophilus NCFM	$4,5x10^9$ UFC	*
Lactobacillus paracasei Lpc-37	$4,5x10^9$ UFC	*
Bifidobacterium animalis subsp lactis BI-04	$4,5x10^9$ UFC	*
Bifidobacterium animalis subsp lactis Bi-07	$4,5x10^9$ UFC	*

Não contém quantidades significativas de valor energético, proteínas, carboidratos, gorduras totais, gorduras saturadas, gorduras trans, fibra alimentar e sódio. *VD não estabelecido.

SUGESTÃO DE USO

ADULTOS (MAIORES DE 19 ANOS), **INGERIR 3 CÁPSULAS AO DIA.**

USO NOTURNO, ANTES DE DORMIR

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CSC

Mascaras naturais

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Review > Skin Therapy Lett. 2017 May;22(3):5-7.

The Role of Skin Care in Optimizing Treatment of Acne and Rosacea

C Zip [1]

Affiliations + expand

PMID: 28492949

Free article

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Abstract

A triad approach to the treatment of acne and rosacea has been recommended. This integrated management approach includes patient education, selection of therapeutic agents, and initiation of an appropriate skin care regime. Proper skin care in patients undergoing treatment of both acne and rosacea includes use of products formulated for sensitive skin that cleanse, moisturize and photoprotect the skin. Both acne and rosacea are associated with epidermal barrier dysfunction, which can be mitigated by suitable skin care practices. Appropriate skin care recommendations for patients with acne and rosacea will be discussed.

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JEADV JOURNAL OF THE EUROPEAN ACADEMY OF DERMATOLOGY & VENEREOLOGY

Original Article | Free Access

A cream containing the sap of oat plantlets and mandarin extract soothes the symptoms of rosacea and improves the quality of life of patients

G. Fabbrocini, C. Beziat Monteil, F. Carballido

First published: 07 July 2022 | https://doi.org/10.1111/jdv.18201 | Citations: 1

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Background

Rosacea is a chronic inflammatory disease of the facial skin that affects all skin types and occurs mostly in adults. The main clinical sign of rosacea is a characteristic and persistent form of centro-facial erythema that is prone to exacerbation and can impair quality of life (QoL). The current therapeutic approach for rosacea is to combine various treatments, use appropriate skincare products and avoid flare-up triggers.

Objective

To evaluate the use of a facial skincare product containing protein-free sap extruded from Rhealba® oat plantlets and mandarin extract in subjects with rosacea.

Methods

Three clinical studies were conducted in adult subjects with various rosacea phenotypes (erythematotelangiectatic or papulopustular) and treatment histories to assess the dermatological and ophthalmological tolerance of the study product, as well as its clinical effectiveness, after a twice-daily application on the whole face and neck for up to 4 weeks.

Results

Tolerance of the product was rated as good to very good by dermatologists across the three studies, which involved a total of 105 evaluable subjects. Subjects with untreated erythematotelangiectatic rosacea reported fewer functional signs and symptoms of the disease and an improved QoL. The evaluation of skin biometric parameters revealed a reduction in transepidermal water loss, indicating that the study product helped to restore skin barrier integrity after 4 weeks, and a higher skin pH, indicating that the cutaneous microbiote was respected. Most subjects (93%) who had either undergone a superficial dermatological procedure for erythematotelangiectatic rosacea or were taking oral/topical treatments for papulopustular rosacea, rated the study product as very good (8/10) and felt it further relieved their symptoms.

Conclusion

Overall, the study product was very well tolerated and may be beneficial for subjects with rosacea as an adjunct to superficial dermatological procedures or oral/topical therapies, in line with the current recommendations for rosacea management.

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Review > J Altern Complement Med. 2017 Dec;23(12):920-929. doi: 10.1089/acm.2016.0398. Epub 2017 Jun 26.

The Role of Polyphenols in Rosacea Treatment: A Systematic Review

Suzana Saric [1], Ashley K Clark [1], Raja K Sivamani [2], Peter A Lio [3], Hadar A Lev-Tov [4]

Affiliations + expand

PMID: 28650692 DOI: 10.1089/acm.2016.0398

Abstract

Objectives: Various treatment options are available for the management of rosacea symptoms such as facial erythema, telangiectasia, papules and pustules, burning, stinging, and itching. Botanical therapies are commonly used to treat the symptoms. The objective of this review is to evaluate the use of polyphenols in rosacea treatment.

Design: PubMed, Embase, Biosis, Web of Knowledge, and Scopus databases were systematically searched for clinical studies evaluating polyphenols in the management of rosacea.

Results: Of 814 citations, 6 met the inclusion criteria. The studies evaluated licochalcone (n = 2), silymarin (n = 2), Crysanthellum indicum extract (n = 1), and quassia extract (n = 1). The studies only evaluated topical formations of stated polyphenols. Main results were summarized.

Conclusions: There is evidence that polyphenols may be beneficial for the treatment of rosacea symptoms. Polyphenols appear to be most effective at reducing facial erythema and papule and pustule counts. However, studies included have significant methodological limitations and therefore large-scale, randomized, placebo-controlled trials are warranted to further assess the efficacy and safety of polyphenols in the treatment of rosacea.

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Clinical Trial > J Clin Pharmacol. 2012 Jan;52(1):84-8. doi: 10.1177/0091270010391533. Epub 2011 Feb 22.

Evaluation of the efficacy and tolerance of a topical gel with 4% quassia extract in the treatment of rosacea

Alicia Ferrari [1], Christian Diehl

Affiliations + expand

PMID: 21343346 DOI: 10.1177/0091270010391533

Abstract

Background: There are various treatment options available for rosacea, depending on the subtype, but treatment is still generally unsatisfactory. Some studies have reported antiparasitic and anti-inflammatory properties of Quassia amara.

Aim: To check the efficacy and safety of a topical gel with 4% Quassia amara extract in the treatment of various grades of rosacea.

Methods: A group of 30 patients with various grades of rosacea (I-IV) were investigated in a single-center, open-label study. They were treated with a topical gel with 4% Quassia amara extract for 6 weeks. Response was evaluated by the flushing, erythema, telangiectasia, papules, and pustules scores. At the end of therapy, overall improvement, safety, and tolerability were assessed.

Results: Twenty-seven of 30 patients (90%) completed the study. The treatment resulted to be very effective, and the results achieved were in line with those published with topical metronidazole and azelaic acid. Safety and tolerability were excellent.

Conclusion: Topical quassia extract could be a new, efficient, and safe weapon in the armamentarium for the management of rosacea.

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Review

New Herbal Biomedicines for the Topical Treatment of Dermatological Disorders

Julia Hoffmann, Fabian Gendrisch, Christoph Mathis Schempp and Ute Wölfle *

Research Center Skinitial, Department of Dermatology, Medical Center, University of Freiburg, Hauptstr. 7, D-79104 Freiburg, Germany; julia.hoffmann@uniklinik-freiburg.de (J.H.); fabian.gendrisch@uniklinik-freiburg.de (F.G.); christoph.schempp@uniklinik-freiburg.de (C.M.S.)

* Correspondence: ute.woelfle@uniklinik-freiburg.de; Tel.: +49-0761-27068250; Fax: +49-0761-27068290

Received: 8 January 2020; Accepted: 6 February 2020; Published: 8 February 2020

check for updates

Abstract: Herbal extracts and isolated plant compounds play an increasing role in the treatment of skin disorders and wounds. Several new herbal drugs, medicinal products and cosmetic products for the treatment of various skin conditions have been developed in recent years. In this nonsystematic review, we focus on herbal drugs that were tested in controlled clinical studies or in scientifically sound preclinical studies. The herbal biomedicines are intended to treat atopic dermatitis (St. John's wort, licorice, tormentil, bitter substances, evening primrose), psoriasis (araroba tree, lace flower, barberry bark, indigo, turmeric, olibanum, St. John's wort), actinic keratosis (birch bark, petty spurge), herpes simplex (lemon balm, sage and rhubarb), rosacea (green tea, licorice, tormentil) and acne vulgaris (tea tree oil, green tea, hop), or to improve photo protection (green tea, Dyer's weed, cocoa tree, carotinoids, licorice), aesthetic dermatology (licorice, pine bark, gotu kola) and wound healing (birch bark, onion).

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J Cosmet Dermatol. 2022 Sep; 21(9): 3759–3767.
Published online 2022 Jul 19. doi: 10.1111/jocd.15189

PMCID: PMC9796497
PMID: 35765796

Integrative skincare trial of intense pulsed light followed by the phyto-corrective mask, phyto-corrective gel, and resveratrol BE for decreasing post-procedure downtime and improving procedure outcomes in patients with rosacea

Sheila C. Barbarino, MD, FACS,[1] Vivian W. Bucay, MD,[2] Joel L. Cohen, MD,[3,4] and Michael H. Gold, MD[5]

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Background

Rosacea is a chronic inflammatory skin condition of varying severity that can significantly impact patient quality of life. Intense pulsed light (IPL) is an established treatment for rosacea-associated telangiectasia, inflammation, and erythema.

This study assessed whether application of a phyto-corrective mask, gel, and resveratrol antioxidant serum after IPL treatment can improve outcomes and reduce procedure-related adverse effects.

Methods

In a prospective, open-label, split-face, 3-month study, 10 subjects with moderate-to-severe facial rosacea underwent IPL treatment on both sides of the face. The following were applied to the right side of the face only: phyto-corrective mask once weekly starting immediately after IPL; phyto-corrective gel twice daily; and resveratrol antioxidant treatment at night. Both sides of the face were treated with sunscreen. Subjects were assessed on Day 1, 1 and 3 months after IPL by three, independent evaluators using the 5-point Global Aesthetic Improvement Scale (GAIS). All subjects rated skin redness, hydration, and overall improvement on Day 1 and completed a patient satisfaction questionnaire at the 1- and 3-month visits.

Results

Ten women were enrolled, aged 44–72 years old, with moderate ($n = 6$) to severe ($n = 4$) rosacea. IPL was effective at reducing symptoms with rosacea classified as absent in five women and mild in five at the final 3-month visit. GAIS scores also revealed improvements on both sides of the face, but the skincare treated side showed continuous improvement over 3 months with all patients remaining at least “Improved”, whereas there appeared to be a waning effect after 1 month with IPL alone. On Day 1 after IPL, all women reported less redness, improved hydration, and improved skin appearance on the right side of the face. Patient satisfaction was consistently rated higher on the right side of the face.

Conclusion

Application of a phyto-corrective mask, gel, and resveratrol antioxidant serum may complement IPL treatment for rosacea by enhancing treatment outcomes and reducing procedure-related symptoms.

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Babosa

Desde os tempos antigos, o gel incolor obtido da folha de aloe barbadensis apresenta benefícios multifacetados. Sendo um dos remédios mais confiáveis para uma variedade de problemas de pele, o gel fresco de aloe vera é crucial no tratamento da rosácea. As potentes propriedades anti-inflamatórias e curativas do aloe vera não apenas diminuem os sintomas da rosácea, mas também conferem um brilho natural à pele. Cubra a parte afetada com gel de aloe vera, para reduzir o inchaço e a inflamação.

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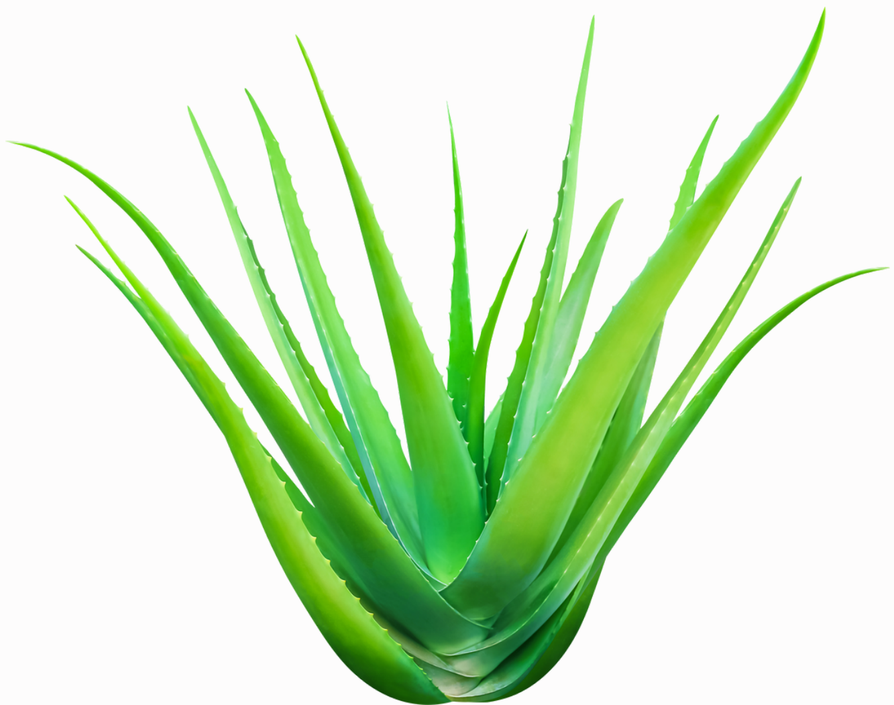

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Cúrcuma

Apresentado como uma potência de propriedades anti-inflamatórias e antioxidantes, este pó amarelo mágico é conhecido por lidar com vários problemas de pele, incluindo hiperpigmentação, sinais de envelhecimento e cicatrizes. O componente ativo curcumina no açafrão ajuda a aliviar a inflamação quando aplicado topicamente ou consumido como alimento.

Aplique uma pasta de açafrão em pó e iogurte na pele afetada para reduzir a vermelhidão e o inchaço.

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Mel

Sendo um limpador natural e imbuído de propriedades antimicrobianas, o mel é extremamente benéfico na prevenção da propagação da infecção por rosácea. Quando aplicado na pele, não só acalma a pele seca e inflamada, mas também previne o crescimento de bactérias causadoras de acne.

Massageie a pele afetada com algumas gotas de mel cru para obter os resultados desejados em quinze dias.

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Chá verde

Um remédio incrível para a rosácea, o chá verde infundido com polifenóis e antioxidantes, não só ajuda a reduzir a inflamação e o inchaço, mas também trata o rubor e a vermelhidão da pele. Também reduz a reação da pele aos raios UVA e UVB e, assim, ajuda a domar os surtos desencadeados pelo sol.

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Aplique o chá verde resfriado na pele afetada pela rosácea usando bolas de algodão para reduzir a vermelhidão e a inflamação.

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Feno grego

Enriquecido com nutrientes saudáveis, como vitaminas A, C, K, ácido fólico, potássio, cálcio e ferro, o feno grego em pó é conhecido por possuir propriedades anti-inflamatórias e antioxidantes. Previne a formação de acne cheia de pus na pele afetada e também reduz manchas vermelhas. Também previne coceira e irritação e acalma a pele inflamada.

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Aplique uma pasta de feno grego em pó misturada com água de rosas nas áreas afetadas da pele para obter alívio da coceira na pele inflamada e obter uma tez natural, suave e brilhante.

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Bardana

A bardana está disponível como suplemento, extrato ou alimento.

Como um remédio comestível, a raiz de bardana pode purificar o fígado e eliminar problemas de pele, como acne. O extrato da planta de bardana pode ser um tratamento eficaz para a rosácea.

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Camomila

Assim como a babosa, a camomila é comum em produtos hidratantes para a pele. É frequentemente usado topicamente no tratamento à base de ervas para a pele inflamada.

Muitos estudos apóiam ela como um remédio bem-sucedido.

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O óleo essencial de camomila também pode ser diluído em creme e aplicado. Você também pode fazer chá de camomila, esperar que esfrie e aplicá-lo como uma lavagem ou compressa na pele.

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Óleo de côco

Hidratantes como o óleo de coco são populares para todos os tipos de doenças inflamatórias da pele, incluindo a rosácea.

Não há estudos recentes mostrando que o óleo de coco é eficaz para a rosácea. Ainda assim, poderia ajudar como um conhecido anti-inflamatório, antioxidante e hidratante.

Aplique uma pequena quantidade de óleo de coco de alta qualidade na pele. Você também pode usá-lo como um óleo transportador para óleos essenciais apropriados, se desejar.

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Confrei

O confrei é uma planta que produz um composto chamado alantoína. Este composto chamou a atenção por aliviar as reações cutâneas.

Um estudo de 2017 sobre um produto contendo alantoína descobriu que ele melhorou os sintomas praticamente sem efeitos colaterais. Também reduziu a vermelhidão e a aparência.

Encontre produtos como cremes, loções ou pomadas que contenham confrei natural ou alantoína e siga as instruções do rótulo ou macere a erva e misture no seu creme.

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Matricária

É outra planta frequentemente nomeada como um remédio natural para a rosácea.

Matricária tem qualidades antioxidantes. Também é conhecido por proteger contra a exposição aos raios UV, como um protetor solar suave. Às vezes, sabe-se que a exposição aos raios UV piora a rosácea.

Certifique-se de usar produtos tópicos de matricária que não contenham partenolídeos. Partenolídeos podem aumentar a sensibilidade da pele. Uso tópico como chá.

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DIA UM

Café da manhã
3 ameixas
2 col. de sopa de sementes de abóbora
1 toca de sopa de sementes de chia moídas
1 col de sopa de quinoa
1 maçã orgânica grande

Almoço
Cereal
Feijão
Peixe
Alface picada, cebola, coentro
Abacate

Lanche da tarde
1 punhado de vegetais crus (cenoura, pepino)

Jantar
Cordeiro grelhado, carne de veado, caça selvagem ou ave
2 col. de sopa de vegetais verdes marinados: feijão verde, aspargos, couve de bruxelas, variedades de couve
cebola roxa grelhada
1 colher de azeite
suco de limão espremido fresco
sal marinho a gosto
10 uvas roxas escuras
Suco de cranberry 100% sem açúcar

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DIA DOIS

Café da manhã

- Iogurte de coco ou kefir
- 1 colher de chá de sementes de linhaça orgânico moídas
- 2 cerejas
- 2 col. de sopa de sementes de girassol
- Suco de açaí
-

Lanche do meio da manhã

- Smoothie verde

Almoço

- 2 coll. de sopa de arroz integral com algas
- 2 col de sopa de pasta de amêndoa
- 1 maçã orgânica grande

Lanche do meio da tarde

- Smoothie verde

Jantar

- 1 c de macarrão de arroz integralsem glúten
- Frango
- Pesto de manjericão
- 2 c de legumes amarelos refogados: cenouras, batata-doce e abóbora
- Suco de romã

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DIA TRÊS

Café da manhã

- 2 ovos com espinafre orgânico refogado
- Azeite para refogar o espinafre
- Fatias de cebola vermelha e alho a gosto
- 1 1/2 col de sopa de mirtilos
- Suco de romã

- **Lanche do meio da manhã**
- 1 c de melão em cubos
- 6 castanhas de caju

- **Almoço**
- Arroz integral cozido, quinoa ou painço
- Legumes verdes cozidos no vapor
- Azeite de oliva
- suco de limão fresco espremido
- Especiarias suaves
- 2 damascos médios

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- **Lanche do meio da tarde**
- Ervilhas ou grão de bico

- **Jantar**
- Folhas verdes cruas e vegetais refogados
- 6 biscoitos de arroz integral pequenos
- Azeite de oliva
- Suco de limão espremido fresco e especiarias suaves
- 2 c sopa de lentilha

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DIA QUATRO

Café da manhã
- 1 col. de sopa de pasta de amêndoa
- 11 col. de sopa de geléia 100% de frutas
- Suco de ameixa com limão

Lanche do meio da manhã
- Smoothie verde

Almoço
- Arroz integral cozido ou quinoa
- Frango grelhado
- Hummus com azeite de oliva
- Suco de limão fresco espremido
- Salsa, cebola, pepino

Lanche do meio da tarde
- Smoothie verde

Jantar
- Pargo selvagem grelhado, sardinha ou salmão
- Legumes verdes marinados grelhados
- Azeite para grelhar peixe e legumes
- Suco de limão espremido fresco, especiarias suaves
- 1 c sopa de abóbora
- Suco de açaí

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DIA CINCO

Café da Manhã

- 1/2 c de cereais de flocos de quinoa cru
- 1 col. de sopa linhaça orgânica moída
- 2 passas ou tâmaras
- Canela
- 4 nozes
- Suco de maçã

Lanche do meio da manhã

- 4 damascos

Almoço

- Folhas verdes cruas e vegetais
- Nozes
- Cerejas ou kiwi
- 8 azeitonas
- Azeite de oliva
- vinagre de maçã cru, sal marinho, cebola vermelha
- 3 ameixas

Lanche do meio da tarde

- 5 castanhas do Brasil

Jantar

- arroz branco com suco de limão e salsa
- feijão preto
- legumes grelhados
- abacate + milho, alface romana picada
- Suco de limão

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Onde comprar?
Site das marcas
Mercado Livre (ver pontuação do vendedor)
Amazon
Iherb

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